UCSF Medical Center
Orthopaedic Institute

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San Francisco, CA 94158
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From: UCSF Orthopaedic Institute	To: GBPT
Phone: (415) 353-4983 Fax: (415) 353-2956	Company:
Date: September 27, 2024	Phone: 209-585-4100 Fax: 209-222-6185
Re: Clinical Notes	Number of Pages: w/ coversheet

Special Instructions/Note:

Please see attached

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Steinhart, Kaitlyn

MRN: 98961003

Office Visit 9/5/2024
UCSF Ortho Spine Redwood Shores

Provider: Patricia Zhong Zheng, MD (Physical Medicine and Rehabilitation)
Primary diagnosis: Radiculopathy of lumbar region
Reason for Visit: New Patient Evaluation • Back Pain; Referred by None Per Patient Provider

Progress Notes

Patricia Zhong Zheng, MD (Physician) • Physical Medicine and Rehabilitation

UCSF Department of Orthopaedic Surgery
Non-Operative Spine Service

Patient ID:
Kaitlyn Steinhart
98961003
26 y.o.
9/19/1997

DATE OF SERVICE: 9/5/2024

Visit Type: New patient

Requesting Provider: Provider, None Per Pati*

CHIEF COMPLAINT

Kaitlyn Steinhart is a 26 y.o. female who presents with a chief complaint of back pain.

HISTORY OF PRESENT ILLNESS

Kaitlyn Steinhart is a 26 y.o. female with a history of L4-S1 fusion for spondylolisthesis. Patient previously had back and leg symptoms before the surgery that completely resolved afterwards. The patient has been in relatively good health until some months ago when back pain going down bilateral legs recurred. Patient does have per record a history of depression and anxiety. Symptoms have been so severe that the primary has ordered MRI of lumbar spine and refer her here for a review. Patient has not had physical therapy for her spine recently. Patient does feel weak in her legs but this has been stable now for some months. At one time in April, she actually thought this was improving though it may have gotten worse since that at that time. She also reports some numbness over her legs. Patient is interested in physical therapy. She would like to avoid injections and surgeries if she can. She does have a pain medicine evaluation scheduled in the upcoming weeks.

PAST MEDICAL HISTORY

Past Medical History:

Diagnosis	Date
• Allergy	
• Anxiety	
• Depression	
• Headache	
• Low back pain *L4-S1 fusion in 2011*	

PAST SURGICAL HISTORY

Past Surgical History:

Procedure	Laterality	Date
• BACK SURGERY		

L4-S1 fusion in 2011
- TONSILLECTOMY

ALLERGIES

Latex, Penicillins, and Diazepam

MEDICATIONS

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• ARIPiprazole (ABILIFY) 10 mg tablet	Take 1 tablet (10 mg total) by mouth daily		
• ascorbic acid, vitamin C, (VITAMIN C) 1,000 mg tablet			
• buPROPion (WELLBUTRIN XL) 300 mg 24 hr tablet	Take 1 tablet (300 mg total) by mouth daily		
• busPIRone (BUSPAR) 10 mg tablet	Take 2 tablets (20 mg total) by mouth 2 (two) times daily		
• gabapentin (NEURONTIN) 300 mg capsule	Take 1 capsule (300 mg total) by mouth every morning, afternoon, and evening.	90 capsule	0
• levonorgestrel (MIRENA INTRAUTERINE)			
• LORazepam (ATIVAN) 1 mg tablet	Take 1 tablet (1 mg total) by mouth		
• meloxicam (MOBIC) 15 mg tablet	Take 1 tablet (15 mg total) by mouth daily		
• ondansetron (ZOFRAN) 8 mg tablet	Take 1 tablet (8 mg total) by mouth every 8 (eight) hours as needed		
• prazosin (MINIPRESS) 1 mg capsule	Take 1 capsule (1 mg total) by mouth nightly at bedtime		
• ARIPiprazole 10 mg tbsp	(Patient not taking: Reported on 9/5/2024)		

No current facility-administered medications for this visit.

SOCIAL HISTORY

Occupation:	Student And Nursery Worker
Marital Status:	Divorced
Tobacco Use:	reports that she has been smoking vape. She has never used smokeless tobacco.

FAMILY HISTORY

Family History

Problem	Relation	Name	Age of Onset
• Mental illness	Mother	Sherrye	

• Diabetes	Father	Kurt
• Depression	Brother	Brandon

Objective:

PHYSICAL EXAM:

Ht 154.9 cm (5' 1") | Wt 68.5 kg (151 lb) | BMI 28.53 kg/m²
General: Pleasant 26 y.o. female in no apparent distress
Pain Diagram
Lumbar spine:
Inspection - Normal lumbar lordosis
Palpation - TTP over the bilateral lumbosacral junction
ROM - Flexion to 20 degrees, extension to 5 degrees
Motor - Overall somewhat difficulties with full effort. With enough coaxing actually close to 5/5 strength with bilateral hip flexion, knee extension, dorsiflexion, plantar flexion.
Sensory - No true loss of sensation to light touch
Gait - Normal, narrow based gait

IMAGING:
I have personally reviewed and interpreted the films available.

XR SCOLIOSIS STUDY 2 OR 3 VIEWS 9/5/2024
CLINICAL HISTORY provided by referring provider: Assess alignment

COMPARISON: Lumbar spine x-ray 2/9/2024, MR lumbar spine 8/3/2024

IMPRESSION:

12 thoracic and 5 non rib-bearing lumbar type vertebral bodies. Last well-formed disc space is L5-S1

Hardware: Posterior spinal fusion hardware spanning L4-S1, in similar alignment compared to prior.

Alignment:
Scoliosis: No scoliosis.
Coronal Imbalance: No coronal imbalance.
Sagittal Imbalance: No sagittal imbalance.
Spondylolisthesis: Similar grade 4 anterolisthesis of L5 on S1.

Vertebral body heights are maintained. Limited evaluation of the upper thoracic vertebral bodies due to overlying structures.

Nonoperative intervertebral disc spaces are maintained.

Intrauterine device overlying the mid pelvis.

MRI Lumbar spine

TURLOCK IMAGING SERVICES

Turlock Imaging Services
3900 Geer Road
Turlock, CA 95382
Office: (209) 669-0600
Fax: (209) 668-4397

PATIENT NAME: KAITLYN STEINHART
PATIENT ID: TU148124
PHONE: 2092778675

BIRTHDATE: 09/19/1997
ALT ID:
DATE OF SERVICE: 08/03/2024

EXAM: TU100270798 MRI, LUMBAR SPINE S/ CONTRAST

REFERRING PHYSICIAN: DIANA REISS, PA
ADDRESS: 1141 NORTE OLIVE AVENUE TURLOCK, CA 95382
CC:

MRI, Lumbar Spine s/ Contrast

EXAM DATE: 8/3/2024 10:45 AM PDT

CLINICAL INDICATION: Low back pain, Other chronic pain

COMPARISON: None.

TECHNIQUE: MRI of the lumbar spine is performed on a high-field magnet without contrast including sagittal and axial T1 and T2 weighted sequences.

FINDINGS:

Alignment: Grade 3 spondylolisthesis of L5 upon S1.
Marrow: No suspect focal marrow lesion.
Pre and paravertebral soft tissues: Unremarkable.
Conus: Normal distal thoracic cord and conus terminating at L1.
Developmental: No developmental spinal stenosis.
Disc spaces:
Lower thoracic: Normal T10, T11 and T12 discs.

L1-L2: Normal.

L2-L3: Normal.

L3-L4: Normal.

L4-L5: Disc is normal. Pedicular screws. Lateral fusions. Posterior bilateral laminectomy.

L5-S1: The disc is absent with fusion of the bodies. Pedicular screws are present posteriorly at L5 and S1. Bilateral wide laminectomy extending through S1. Lateral fusions appear solid.

TURLOCK IMAGING SERVICES
PATIENT NAME: KAITLYN STEINHART
PATIENT ID: TU148124

OFFICE: (209) 669-0600
BIRTHDATE: 09/19/1997
ALT ID:
DATE OF SERVICE: 08/03/2024

EXAM: TU100270798 MRI, LUMBAR SPINE S/ CONTRAST

IMPRESSION:

1. Status post pedicular screw fusions and lateral fusions, L4-S1 with wide laminectomy beginning at L5 extending through S1.
2. Grade 3 spondylolisthesis of L5 upon S1. Likely bony fusion. No residual disc material.
3. No acute abnormality is apparent.

Read By: Craig Fetz, MD
Dictated on: 8/5/2024 9:02 AM PDT
Electronically Signed By: Craig Fetz, MD
Signed on: 8/5/2024 9:11 AM PDT
Workstation Name: TISRAD00002

Service performed during Public Health Emergency (42 U.S.C 247d-6d) PPE supplies utilized according to CDC guidelines.

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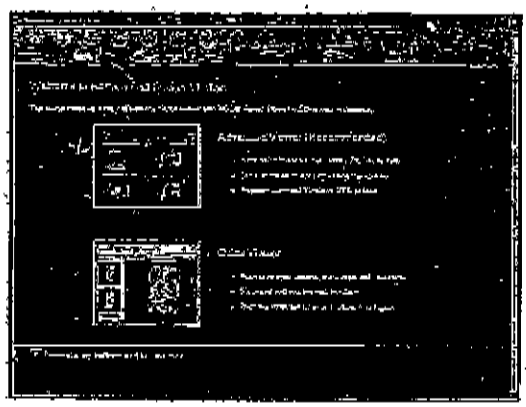
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View Documents-patients reports
Help .
Annotate- mark on exams
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To **EXIT** exam click on the **CLOSE** button in top left corner
To **EXIT** program click on "user,user" in top right corner and select **QUIT**

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You can return the main page by selecting the Return button in the top right corner of the page or to exit the program simply click on the X in the top right corner of the window.

Assessment & Plan:

Kaitlyn Steinhart is a 26 y.o. female with a history of L4 to S1 fusion for spondylolisthesis in 2011in Colorado. Patient reports that for some years after the surgery, she had complete resolution of back and leg symptoms. However, in the last year or so, patient has had recurrence of back pain going down both legs. There is some weakness noted on exam today but some of this may be pain inhibition as well as numbness. Overall, MRI lumbar spine was generally reassuring. I do see perhaps some residual bilateral L5-S1 neural foraminal stenosis. I see some adjacent segmental disease but there is no severe narrowing on top of the fusion level. I encouraged to return to physical therapy. Patient is also very interested in aquatic therapy. She is on meloxicam and I provided gabapentin. I also got a scoliosis x-ray to look for

overall alignment.

Medical decision making:

PLAN:

The natural history as well as any appropriate treatment options were discussed with the patient today. At this time, the treatment plan includes:

1. IMAGING: I got x-rays of scoliosis and to look for overall alignment.
2. THERAPY: I encouraged patient to work on core and hip girdle stabilization and made some local therapy recommendations. She would also benefit from aquatic therapy which she is interested in.
3. MEDICATIONS: Patient has been placed on meloxicam which she finds helpful. I encouraged patient to take this with food and adequate hydration. I prescribed gabapentin. Patient was instructed to start with 300mg qHS. We discussed side effects including somnolence, nausea, fogginess, and increased risk of falls. Should gabapentin be tolerated, She can increase gabapentin to 300mg qAM and qHS after 5 days and 300mg TID after 5 more days. Patient was instructed to not stop the medication abruptly given seizure risks. She agreed to try the medication given its potential benefit.
4. FOLLOW UP: Patient advised to follow up with the pain management provider across her home. They can trial epidural steroid injection and possibly even blocks and sacroiliac joint steroid injections for diagnostic and therapeutic purposes. Patient can return to me if I can be of assistance. I did ask her to seek more urgent care should weakness or numbness worsen. An indication for surgery would be if there is acute worsening of numbness and weakness with evidence of nerve compression on imaging.

I, Abu Turab Mohammed Fakhrouddin am acting as a scribe for services provided by Patricia Zhong Zheng, MD on 09/06/24, 9:51 AM.

The above scribed documentation accurately reflects the services I have provided.
Patricia Zhong Zheng, MD
9/6/2024 9:22 PM

Billing was based on MDM:
Complexity: High (1 acute/chronic illness with neurologic changes - weakness or sensory loss)
Data: Extensive (3+ independent personal interpretation of diagnostic tests and/or reviewed external notes and/or reviewed/ordered 3 tests and/or obtained history from family member)
Risk: Moderate (prescription drug management and counseling, decision regarding referrals and/or procedures/diagnostic testings)

I explained that I am happy to serve as the continued focal point for her non-operative spine needs and coordinate appropriate care with therapists and primary care.

Patricia Zheng, MD
Non-operative Spine Service
Department of Orthopaedic Surgery
University of California, San Francisco

Instructions

Gabapentin: I have prescribed you gabapentin, a nerve pain agent. Please start with 1 tab at night before bedtime. Gabapentin can cause sedation and mood changes. If this is unbearable, please stop. If you are doing well with 1 tab at night, you can add 1 tab in the morning (so 1 tab in the morning and 1 tab at night) after 5 days. If you're doing well, you can add 1 tab after lunch (so 3 tabs a day). Keep the 3 tabs a day until we next see each other (that's 1 tab three time a day). Do not stop the medication abruptly given seizure risks.

Consider bilateral L5-S1 transforaminal epidural steroid injection with your Pain Management group.

Start physical therapy closer to home. Look into water therapy. If you need a specific referral for the aquatic therapy, then please contact me on MyChart.

AMB Complete AVS (Automatic SnapShot taken 9/6/2024), Immunizations (Automatic SnapShot taken 9/6/2024)

Additional Documentation

Vitals: Ht 154.9 cm (5' 1") Wt 68.5 kg (151 lb) BMI 28.53 kg/m² BSA 1.72 m² Pain Sc 8 (Loc: Back)

Flowsheets: Patient-Reported Data, Patient Assessment

SmartForms: UCSF AMB PATIENT ASSESSMENT FORM •
AMB PATIENT EDUCATION LEARNING ASSESSMENT ADVANCED

Standardized Questionnaire 2252 4099

Question	8/29/2024 2:10 PM PDT - Filed by Patient
We are asking these questions of all patients to better understand our patients' sexual and reproductive health needs, and to improve the health of our sexual and gender minority patients.	
Patient's chosen first name (if different than legal name)	Kaity
Patient's pronouns:	she/her/hers
Please select the option that best represents your gender identity:	Female
What sex is listed on your original birth certificate?	Female
Do you think of yourself as:	Straight
How do you learn best? (or your family/caregiver)	Picture/Video
Do you have any barriers to learning? (or your family/caregiver)	No Barriers
Do you have any cultural or religious practices or spiritual beliefs that we should be aware of?	No
Who do you live with?	Significant other
What is your current Marital Status?	Divorced
What is your current Employment Status?	Student - Full Time
What is your Occupation?	Student and nursery worker
How often do you have a drink containing alcohol?	Monthly or less
How many drinks containing alcohol do you have on a typical day when you are drinking?	1 or 2
How often do you have six or more drinks on one occasion?	Never
How hard is it for you to pay for the very basics like food, housing, medical care, and heating?	Somewhat hard
Within the past 12 months, you worried that your food would run out before you got the money to buy more.	Never true
Within the past 12 months, the food you bought just didn't last and you didn't have money to get more.	Never true
In the past 12 months, has lack of transportation kept you from medical appointments or from getting medications?	No
In the past 12 months, has lack of transportation kept you from meetings, work, or from getting things needed for daily living?	No
In the past 12 months has the electric, gas, oil, or water company threatened to shut off services in your home?	No
In the last 12 months, was there a time when you were not able to pay the mortgage or rent on time?	No
In the last 12 months, how many places have you lived? (range: at least 0)	1

Question	8/29/2024 2:10 PM PDT - Filed by Patient
In the last 12 months, was there a time when you did not have a steady place to sleep or slept in a shelter (including now)?	No

Orders Placed

XR Scoliosis Study 2 or 3 Views (Resulted 9/5/2024)
Ambulatory Referral to Physical Therapy New Request

Medication Changes

As of 9/5/2024 2:05 PM

	Refills	Start Date	End Date
Added: gabapentin (NEURONTIN) 300 mg capsule Take 1 capsule (300 mg total) by mouth every morning, afternoon, and evening. - Oral	0	9/5/2024	10/5/2024

Medication List at End of Visit

As of 9/5/2024 2:05 PM

	Refills	Start Date	End Date
aripiprazole			
ARIPiprazole (ABILIFY) 10 mg tablet Take 1 tablet (10 mg total) by mouth daily - Oral Patient-reported medication	—	6/6/2024	—
ARIPiprazole 10 mg tbsp Patient-reported medication	—		—
ascorbic acid, vitamin C, (VITAMIN C) 1,000 mg tablet Patient-reported medication	—		—
buPROPion (WELLBUTRIN XL) 300 mg 24 hr tablet Take 1 tablet (300 mg total) by mouth daily - Oral Patient-reported medication	—		—
busPIRone (BUSPAR) 10 mg tablet Take 2 tablets (20 mg total) by mouth 2 (two) times daily - Oral Patient-reported medication	—		—
gabapentin (NEURONTIN) 300 mg capsule Take 1 capsule (300 mg total) by mouth every morning, afternoon, and evening. - Oral	0	9/5/2024	10/5/2024
levonorgestrel (MIRENA INTRAUTERINE) Patient-reported medication	—		—
LORazepam (ATIVAN) 1 mg tablet Take 1 tablet (1 mg total) by mouth - Oral Patient-reported medication	—		—
meloxicam (MOBIC) 15 mg tablet Take 1 tablet (15 mg total) by mouth daily - Oral Patient-reported medication	—		—
ondansetron (ZOFRAN) 8 mg tablet Take 1 tablet (8 mg total) by mouth every 8 (eight) hours as needed - Oral Patient-reported medication	—	9/13/2023	—
prazosin (MINIPRESS) 1 mg capsule Take 1 capsule (1 mg total) by mouth nightly at bedtime - Oral Patient-reported medication	—	6/6/2024	—

Visit Diagnoses

Primary: **Radiculopathy of lumbar region** M54.16

No. 5397 P. 12/12 Sep. 30. 2024 10:42AM